Nursing Essentials

Priority: Bleeding in later pregnancy: assess maternal stability and fetal status; avoid digital vaginal examination until previa is excluded.

FOCUS 1

Locate

- **Location problem:** placental tissue overlies the cervical opening.
- **Classic pattern:** painless vaginal bleeding with a relatively soft uterus; presentation is not always textbook.
- **Assessment:** ultrasound establishes placental location. Transvaginal ultrasound may be used by the trained team.

FOCUS 2

Recognize loss

- **Separation problem:** the placenta separates before birth.
- **Possible findings:** pain, uterine tenderness / hypertonicity, bleeding, contractions, or fetal compromise.
- **Concealed bleeding:** shock can be out of proportion to visible loss. Ultrasound does not reliably exclude abruption.

FOCUS 3

Stabilize

- **Escalate:** quantify visible bleeding, trend vital signs / perfusion, and assess fetal status.
- **Prepare:** intravenous access, ordered laboratory studies, blood availability, and birth / resuscitation resources.
- **Monitor:** ongoing loss, pain, uterine activity, urine output, and signs of coagulopathy.

Keep the distinctions clear

Make it stick: “Painless versus painful” helps recall the usual patterns, but neither pain nor visible blood alone can confirm or exclude the diagnosis.

Focus	What to know	Nursing application
No digital exam with suspected previa	cervical manipulation can provoke severe hemorrhage.	Avoid digital examination
Abruption	assess for hypovolemia and disseminated intravascular coagulation, including concealed loss.	Assess for concealed loss
Birth plan	depends on maternal / fetal stability, bleeding, gestational age, and placental location—not one symptom alone.	Prepare the prescribed plan

RED FLAGS · ESCALATE PROMPTLY

Heavy or ongoing bleeding, shock, a rigid / tender uterus, or fetal deterioration needs urgent escalation; the visible loss may underestimate severity.